

1. PATIENT PROFILE

Master registry demographics and baseline comorbidities

HF ID

HF00006

CARE MR NO.

MR00004

Visit Type☐ Inpatient☒ Outpatient☐ Home

PATIENT NAME

test

GENDER

Male

AGE & DATE OF BIRTH

40 years / 1985-10-28T18:30:00.000Z

PHONE NO.

7657657342

ADDRESS

Highest Education Level☒ Primary☐ Secondary☐ Graduate☐ Post Graduate☐ None**Monthly Income**

10000.00

Occupation

farmer

Caregiver Name

supriya

Relationship to Patient

wife

Caregiver Phone No.

9876543210

Insurance / Payment Mode☐ Private Insurance☐ Government Reimbursement☐ Arogyasree☐ Direct**Date of Visit / Hospitalization**

19-07-2026

Date of Discharge

08-07-2026

Treating Cardiologist

Dr. G. Sundararaman

**Referring Doctor**

Dr. Rajesh Iyer

**Referred From (Department / Practice)**

E.g. Cardiology Department / Community Clinic

Present Diagnosis

community clinic

**2. INPATIENT DETAILS**

Precipitating factors and admission context for heart failure hospitalizations

If admission for heart failure, please select precipitating factors for admission:

☐ Myocardial ischemia	☐ Atrial fibrillation
☐ Bradyarrhythmia	☐ Ventricular tachycardia
☐ Uncontrolled hypertension	☐ Infection
☐ Renal failure	☐ Anaemia
☐ Medication non adherence	☐ Excessive salt intake
☐ Excessive water ingestion	☐ Progressive worsening

Other Precipitating Factor

Specify other precipitating factors if any

If admission for reasons other than heart failure, please specify reason(s):

E.g., Elective procedure, trauma, etc.

No. of days hospitalized

Number of days

Note: Update LOS & date of discharge upon receiving info.

3. INITIAL CLINICAL ASSESSMENT

Patient background, prior hospitalizations, vitals layout and diagnostic signs

Previous Diagnosis

Specify historical diagnostic parameters...

Medical History

☐ CABG	☐ PTCA
☐ Stroke	☐ Major Bleed
☐ Thrombolysis	

☐ Past MI

Others (Specify separate medical histories)

E.g. Dyslipidemia, PVD

Recent Hospitalization(s)

History of hospitalization for heart failure:

☐ Yes

☒ No

VT/VF Risk Assessment

☐ Documented episode of VT/VF

☐ Complaints of Syncope / Pre-syncope

☐ Documented PVCs

☐ Documented NSVT

Vitals Metrics

Weight (kg) *

70.00

☐ Unable to weigh (Measure at earliest opportunity)

Height (Cm)

170.00

CALCULATED BMI

24.2

Heart Rate (resting) (Bpm) *

72

Variability

☒ Regular

☐ Irregular

Respiratory Rate

18

O₂ Saturation (%)

98.00

Blood Pressure: Sitting / Supine (mmHg)

120

/

80

Blood Pressure: Standing (mmHg)

115

/

76

Mental Status



Alert / Oriented



Confused



Drowsy

Symptoms

Dyspnea at rest:



Yes



No

Weight Loss:



Yes



No

Dyspnea with exertion:



Yes



No

Syncopy:



Yes



No

Fatigue:



Yes



No

PND:



Yes



No

Orthopnea:



Yes



No

Muscle Cramps:



Yes



No

Loss of appetite/Bloating:



Yes



No

Wheeze:

☐ Yes

☒ No

Decreased exercise tolerance:

☐ Yes

☒ No

Giddiness:

☐ Yes

☒ No

Weight Gain:

☐ Yes

☒ No

Other:

☐ Yes

☒ No

Clinical Signs of Volume Overload

Peripheral edema:

☐ Yes

☒ No

Ascites:

☐ Yes

☒ No

Rales:

☐ Yes

☒ No

Jugular venous pressure:

☐ Yes

☒ No

Hepatomegaly:

☐ Yes

☒ No

Other:

☐ Yes

☒ No

4. FINAL CLINICAL ASSESSMENT

Discharge classification, structural etiologies, and outcomes layout

Type of Heart Failure *

☐ HFrEF (HF with reduced EF)

☐ HFpEF (HF with preserved EF)

HF Etiology

Cardiovascular

Non-cardiac

☐ Ischemic

☐ Toxic (Alcohol, Cocaine, Chemotherapeutic)

☐ Idiopathic (Dilated)

☐ Hypertrophic

☐ Restrictive (Amyloid, Sarcoid)

☐ Valvular/ Rheumatic heart disease

☐ Hypertensive

☐ Tachycardia Induced

☐ Metabolic Diseases/Hemochromatosis/Wilson's Disease

☐ HIV and Viral Cardiomyopathy

☐ Inflammatory cardiomyopathy

☐ Reduced EF with/wo previous MI

☐ Pregnancy

☐ Thyroid Disease

☐ Pheochromocytoma

☐ Chronic Renal Disease

Pulmonary

☐ Cor Pulmonale secondary to COPD

☐ Pulmonary Hypertension

Others (please specify):

Specify other etiology...

Comorbidities & Risk Factors

Comorbidities

☐ Associated CAD

☐ Anemia

☐ Renal Failure

☐ CVA

☐ Diabetes Mellitus

☐ Severe musculoskeletal disease

☐ Hypertension

Risk Factors

☐ Smoking

☐ Alcohol

Others:

☐ Cancer

☐ Valvular Disease

☐ APD

☐ Asthma

☐ Bleeding diathesis

☐ COPD

☐ PVD

☐ OSA

Others:

Stage of HF *

☐ Stage A

☐ Stage B

☒ Stage C

☐ Stage D

Functional Status *required *

☐ NYHA Class I

☐ NYHA Class II

☐ NYHA Class III

☐ NYHA Class IV

AF Status *required *

☐ Permanent

☐ Paroxysmal

☐ Persistent

☒ NSR

Major Adverse Cardiac Events

☐ Hospitalization

☐ Major Procedures:

☐ Stroke

☐ Other:

☐ Major Bleed

☐ Severe Arrhythmia

☒ **Death:**

Date:

30-07-2026

Location:

☒ Home ☐ Hospital

Reason:
stroke

Final Clinical Notes / Summary

stroke

5. INVESTIGATIONS

Complete clinical test records, laboratory metrics, and physiological studies grid mapping

ECG *required

Date of test: 29-06-2026 QRS duration: 0.00

Rhythm

- ☐ Sinus
☐ AF
☐ Other:

AV Conduction

- ☐ Normal
☐ 1st degree AV block
☐ 2nd degree AV block
☐ 3rd degree AV block

Q Waves

- ☒ Yes
• Leads: stroke
☐ None

Blockages

- ☒ LBBB
☐ RBB
☐ Other:

Extra Beats

- ☐ APC
☐ VPC
☐ None

QT Interval

QT: 11.00
QTC: 12.00

Chest X-ray

Date of test: —

CT ratio: 0.00

☒ Pleural effusion

☒ PVH

Others:

☒ Pulmonary edema

ECHO

Date of test: 15-07-2026

☒ EF%: 11.00

☒ E/E' ratio: 16.00

☒ E/A ratio: 13.00

☒ E deceleration time: 13.00

☒ RV TAPSV: 15.00

☒ Left Atrium Dimension

☒ Left Ventricle Systole

☒ Left Ventricle Diastole

Other Valves: 67

☐ MR mitral regurgitation:
☐ None ☐ 1plus ☐ 2plus
☐ 3plus ☐ 4plus

RV Systolic Pressure: 45
RV Function: ☐ Normal ☒ Impaired
RWMI:
☐ None ☐ Global ☐ Inferior ☐ Anterior
☐ Lateral

Holter/ Event Recorder

Date of test: 16-07-2026

☒ VPC:

Ventricular Arrhythmia: ☐ No ☐ Yes ☒ Complex VPC ☐ NSVT ☐ VT

Atrial Arrhythmias: ☐ None ☒ APCs ☐ AF

Heart rate variability: 456

Stress Test

Date of test: 27-06-2026

☐ Not Done
☒ Done

▪ METS achieved: 45.00 ▪ Ischemic changes: ☐ Yes ☒ No
▪ Target heart rate achieved: 34 ▪ Arrhythmias: ☒ Yes ☐ No

MRI

☒ LVEF: 13.00 ☐ Scar Present ☐ Scar Absent Date of test: 14-07-2026

PET

Date of test: —

6-Minute Walk Test

Date of test: 29-06-2026

☒ Done:

▪ Distance walked in m: 13.00 ▪ Heart rate recovery in first 1 minute: 15

☐ Not Done, Reasons:

Anaerobic Threshold

Date of test: 07-07-2026

Angiogram

Date of test: 29-06-2026

☒ Done:

☐ Normal

☐ 1 vessel disease

☐ 2 vessel disease

☐ 3 vessel disease

☐ LMCA

☐ Not Done

Endomyocardial biopsy

Date of test: 01-07-2026

☒ Done ☐ Not Done

Vaccinations

☒ Pneumococcal (Date of test: 08-07-2026) ☒ Influenza (Date of test: 06-07-2026)

Blood group

☐ A+ ☐ A- ☐ B+ ☐ B- ☐ AB+ ☒ AB- ☐ O+ ☐ O-

Lab Tests

(*Potassium and Creatinine tests are required for all follow-up visits)

PLEASE SPECIFY:	RESULTS	DATE	WHEN REQUIRED:	RESULTS	DATE
☒ Potassium*	4.8	28-06-2026	☐ BNP		—
☒ Creatinine*	6.3	27-06-2026	☐ NT-pro BNP		—
☐ Hb		—	☐ LDL		—
☐ Calcium		—	☐ INR		—
☐ BUN		—	☐ ST2		—
☐ Glucose		—	☒ hy	3.2	30-06-2026
☐ HBa1c		—			
☐ Magnesium		—			
☐ Sodium		—			
☐ TSH		—			
☐ T3		—			
☐ T4		—			

6. MEDICAL THERAPY (DOSE & FREQUENCY)

Guideline-directed medical therapy with dosing details

RECOMMENDED CONSULTS

Please specify...

DRUG INTOLERANCE(S) / CONTRAINDICATIONS

Please specify...

BETA-BLOCKER

- ☐ Carvedilol
- ☐ Bisoprolol
- ☐ Metoprolol Succinate
- ☐ Nebivolol
- ☐ Other:

CONTRAINDICATION/REASON NOT USED:

- ☐ Bradycardia
- ☐ COPD/ Asthma
- ☐ Other
- ☐ Heart blocks
- ☐ Hypotension

ACE INHIBITOR

- ☐ Enalapril
- ☐ Ramipril
- ☐ Lisinopril
- ☐ Perindopril
- ☐ Other:

CONTRAINDICATION/REASON NOT USED:

- ☐ Elevated creatinine (Renal failure)
- ☐ Cough
- ☐ Other
- ☐ Hyperkalemia
- ☐ Hypotension

ANGIOTENSIN RECEPTOR BLOCKER

- ☐ Valsartan
- ☐ Losartan
- ☐ Telmisartan
- ☐ Olmesartan
- ☐ Other:

CONTRAINDICATION/REASON NOT USED:

- ☐ Elevated creatinine (Renal failure)
- ☐ Hypotension
- ☐ Hyperkalemia
- ☐ Others

ALDOSTERONE ANTAGONIST

☐ Spironolactone☐ Eplerenone

CONTRAINDICATION/REASON NOT USED:

☐ Hyperkalemia☐ Elevated creatinine (Renal failure)☐ Hyponatremia☐ Other**HYDRALAZINE**☐**NITRATE**☐☐**ANTICOAGULATION**☐ Warfarin☐ Vitamin K Inhibitor☐ NOAC☐ Acitrom☐ UFH☐ LMWH**ANTI-PLATELET**☐ Aspirin☐ Clopidogrel☐ Prasugrel☐ Ticagrelor**ANTIARRHYTHMIC**☐ Amiodarone☐ Other:**DIURETIC**☐ Furosemide☐ Torsemide☐ Metolazone☐ Other:

CONTRAINDICATION/REASON NOT USED:

☐ Hyponatremia	☐ Hypokalemia
☐ Worsening renal failure	☐ Hypotension
☐ Other	

DIGOXIN

☐

IVABRADINE

☐ Ivabradine

STATINS

☐ Atorvastatin

☐ Simvastatin

☐ Rosuvastatin

ANTIDIABETICS

☐ Sulfonylureas

☐ Metformin

☐ Glitazone

☐ Gliptin

☐ Acarbose Derivative

☐ Human Insulin

☐ Synthetic Insulin

OTHER

☐ Anti-hypertensive

☐ Thyroxine

☐

☐

☐

☐

\n\n

7. DEVICE THERAPY

Current implanted devices and eligibility assessment

CURRENT DEVICE THERAPY

☐ No Device

☐ Yes, Has Device

ELIGIBILITY FOR DEVICE THERAPY

☐ Not Eligible

☐ Eligible

DATE OF IMPLANT

dd-mm-yyyy

ICD THERAPY ADMINISTERED

☐ ICD Shock

☐ ATP

TECHNICAL LOG PARAMETERS

BiV pacing (%)

E.g. 98.5

AFib burden

E.g. 2% or 10 min/day

NSVT episodes (#)

SVT episodes (#)

Device Volume alert

Details of any volume/fluid alerts

NOTES

Additional device notes...

8. PATIENT EDUCATION

Counseling topics documented for this visit

RECOMMEND PATIENT EDUCATION

☐ Diet: 2000-mg salt restriction

☐ Exercise/activity promoted

☐ Daily weight monitoring

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15/17

- ☐ Disease process explained
- ☐ Smoking cessation
- ☐ Alcohol cessation
- ☐ Medication compliance
- ☐ What to do for worsened symptoms
- ☐ Device therapy education
- ☐ Other

9. RECOMMENDATIONS

Discharge plan, follow-up and therapeutic guidance

☒ FLUID AND DIET

hello

☒ EXERCISE

Yes

☒ YOGA

Yes

☒ SMOKING CESSATION

hello

☒ STRESS MANAGEMENT

hello

☒ DRUGS

hello

☒ INVESTIGATIONS

Yes

☒ PROCEDURES

Yes

☒ OTHERS

hello